Hackensack
Meridian *Health*

Via Electronic Submission - http://www.regulations.gov
The Honorable Mehmet Oz
Administrator
Centers for Medicare and Medicaid Services
U.S. Department of Health and Human Services
Hubert H. Humphrey Building
200 Independence Ave., SW
Washington, D.C. 20201

June 16, 2025

RE: CMS-0042-NC, Request for Information; Health Technology Ecosystem

Dear Administrator Oz,

On behalf of Hackensack Meridian *Health* (HMH), we thank you for the opportunity to comment on the Health Technology Ecosystem Request for Information (RFI) issued by the Centers for Medicare & Medicaid Services (CMS) and Assistant Secretary for Technology Policy (ASTP)/Office of the National Coordinator for Health Information Technology (ONC) (ASTP/ONC).[1]

HMH is the largest, most comprehensive, and truly integrated health care network in New Jersey, comprising a network of hospitals that includes three academic medical centers, one university teaching hospital, two children's hospitals, nine community hospitals, a behavioral health hospital, three rehabilitation hospitals and one long-term acute care hospital. Six of our 18 hospitals maintain robust academic medical programs. HMH also has more than 500 patient care locations, including ambulatory care centers, surgery centers, home health services, long-term care and assisted living communities, ambulance services, lifesaving air medical transportation, fitness and wellness centers, rehabilitation centers, urgent care centers and physician practice locations. HMH has more than 40,000 team members and more than 7,000 physicians within its network and is a distinguished leader in health care philanthropy, committed to the health and well-being of the communities it serves.

Overview of HMH's Comments

HMH strongly supports the goals outlined in the CMS and ASTP/ONC RFI on digital health, particularly its focus on leveraging innovation to enhance patient care, streamline administrative processes, and promote efficiencies. As a leading integrated health network actively deploying artificial intelligence (AI) and digital tools across clinical and operational settings, HMH recognizes the immense opportunity to improve outcomes for millions of Medicare beneficiaries. We believe that digital health innovations can be used to reduce provider burden and drive higher-quality, more personalized care. Our comments reflect real-world experiences in using AI and digital technologies to transform diagnostics, surgery, care coordination, and administrative workflows.

To unlock digital health's full potential, HMH supports policies prioritizing seamless data interoperability and FHIR (Fast Healthcare Interoperability Resources) expansion for secure

[1] 90 Fed. Reg. 21034 (May 16, 2025).

information exchange. We also encourage investment in cloud-based platforms and APIs promoting scalable integration. Policymakers should expand Medicare coverage for clinically validated digital health technologies, simplify reimbursement pathways for providers, and promote the use of patient-centered design to ensure accessibility, especially for older adults. Additionally, supporting AI and predictive analytics is critical to delivering proactive, personalized care recommendations that drive better health outcomes. Our recommendations focus on four key priorities: (1) advancing patient-centered digital tools; (2) removing barriers to interoperable, accessible AI; (3) strengthening provider enablement and workflow integration; and (4) establishing robust frameworks for AI reimbursement, liability, and data privacy.

Section B - Patients and Caregivers

1. PC-3 & PC-5. Patient Needs, Health Management Apps, and CMS Role in Digital Health Products

HMH uses AI to support personalized care pathways that reduce hospital readmissions and improve chronic condition management. For example, AI-powered tools assist in identifying patients at risk of nonadherence to medications, and help caregivers manage care plans with greater clarity and predictability. These tools have demonstrated value in supporting Medicare beneficiaries with complex needs and should be incentivized through reimbursement models and digital quality measures that recognize patient engagement, care navigation, and caregiver relief.

Many patients and caregivers experience friction due to poor interoperability, identity authentication hurdles, and digital literacy gaps.[2] While CMS should not directly endorse or require use of digital products for clinical use, HMH believes that CMS can support the digital health ecosystem by advancing user-friendly, secure digital identity solutions that are interoperable across systems and easy to adopt. Federal policy should ensure these solutions support seamless access to AI-enabled care tools across provider networks and are based on transparent criteria such as effectiveness in improving outcomes and accessibility to seniors.

HMH also sees significant value in AI-powered virtual assistants that can help Medicare beneficiaries and their families understand their care pathways, manage complex medication regimens, and access timely support for symptom management. These tools not only empower patients to take a more active role in their health but also free up provider resources for higher-value clinical activities. CMS can accelerate adoption by establishing clear efficacy and safety benchmarks for digital health products, incentivizing integration into care workflows, and supporting reimbursement for digital interventions that demonstrate improved outcomes and reduced caregiver burden.

Section C - Providers

1. PR-1 & PR-2. Digital Health Product Integration and Obstacles for Providers

From HMH's experience, provider adoption of digital health tools, particularly those powered by AI, is constrained by several interrelated barriers. Chief among them are:

---

[2] https://www.healthit.gov/sites/default/files/page/2023-02/2022_ONC_Report_to_Congress.pdf

- Limited interoperability with electronic health record (EHR) systems, leading to data silos and redundant workflows;
- Absence of clear legal frameworks for liability when clinical decisions are informed by AI outputs; and
- Lack of consistent reimbursement models for AI-enabled care delivery.

Additionally, providers express hesitancy when AI systems lack explainability or clinical validation. To address this, HMH urges CMS to support the development of standardized compliance frameworks for AI developers, such as those based on the NIST AI Risk Management Framework, to ensure that AI tools are validated, explainable, and trustworthy.[3]

For example, HMH has implemented real-time AI analysis of mammograms to detect early signs of breast cancer, resulting in faster diagnoses and improved survival rates. However, scaling such innovations requires clear federal guidance on liability—ensuring accountability for AI developers—and predictable coverage and reimbursement for AI-enabled medical applications. CMS should also encourage independent audits and certifications for AI tools, enabling providers to make informed procurement decisions and reducing the risk of harm from unvalidated or biased algorithms.

2. PR- 2 & PR-7. Provider Workflows, Burnout, and Responsible AI Use

At HMH, digital health tools have been pivotal in reducing clinician administrative burden and optimizing workflow. For example, HMH uses AI-driven documentation assistants that capture real-time clinical notes and integrate directly into the EHR, significantly decreasing time spent charting. Robotic surgery platforms, enhanced by AI, improve procedural precision while reducing clinician fatigue and patient recovery time. HMH also leverages AI to streamline clinical decision-making and manage patient flow. These solutions not only reduce burden but also contribute to better outcomes. CMS should consider exploring ways to account for the measurable impact of such tools on provider efficiency and well-being. Incentivizing AI applications that demonstrably reduce screen time, documentation hours, and rework would accelerate their deployment and uptake.

To integrate AI and digital tools more deeply into care delivery, HMH recommends that CMS consider a multi-pronged policy approach:

- Establish standardized validation and certification pathways for clinical AI tools to ensure safety and transparency;
- Provide specific billing guidance and reimbursement pathways for AI-enabled services such as diagnostics, virtual assistants, and clinical decision support systems;
- Promote API-based EHR integrations using FHIR-native designs to enable plug-and-play functionality across digital tools; and
- Fund ongoing training and continuing education for clinicians using AI tools, alongside liability protections for good-faith use of certified systems.

Such policies would collectively reduce risk, improve interoperability, and encourage sustained use of digital innovations in everyday care.

---

[3] https://www.nist.gov/itl/ai-risk-management-framework

3. PR- 9. Data Privacy.

To effectively balance patient privacy with access to digital health products and services that offer transformative health benefits, CMS must modernize existing privacy frameworks to reflect the unique risks and realities of AI-driven care. Current standards, such as those under HIPAA, were not designed to address the complexities of high-dimensional data use in AI applications. As AI systems increasingly rely on large datasets, including data that is technically de-identified, there is a growing risk that advancements in computing power could allow re-identification of individuals. HMH strongly recommends modernizing HIPAA to explicitly address AI-related data flows, clarify requirements for managing re-identification risk, and establish clear guidelines that permit responsible data use while upholding strong privacy protections. These updates are critical to ensuring that patients continue to benefit from AI-powered tools, such as predictive models, ambient technologies, and personalized clinical decision support, without sacrificing the confidentiality of their personal health information.

HMH further recommends that CMS take a proactive approach to safeguarding patient trust by requiring enhanced privacy safeguards for AI developers. This includes implementing stronger requirements for managing sensitive and hard-to-de-identify data types, such as voice recordings or behavioral biometrics. For AI vendors using de-identified data in product development, privacy frameworks should go beyond current "Safe Harbor" and "Expert Determination" methods, requiring adherence to responsible data use agreements and audit mechanisms. In parallel, patients should have transparent opt-in or opt-out choices regarding the use of their de-identified data for AI training or research. This consumer choice framework would respect patient autonomy and ensure that the benefits of digital health innovation are delivered in a way that is ethically sound and publicly trusted. Taken together, these measures strike an appropriate balance between enabling innovation and preserving the privacy rights of Medicare beneficiaries.

Conclusion

HMH strongly supports the Administration's vision to modernize health care through digital innovation, recognizing that prevention, wellness, and AI-enabled care are essential to building a healthier America. We applaud efforts by CMS and ASTP/ONC to enhance infrastructure, streamline data exchange, and reduce administrative burden. AI and digital tools can play a vital role in improving outcomes, promoting access, and driving smarter care delivery. HMH would welcome the opportunity to serve as a resource and look forward to continued collaboration to ensure the responsible, patient-centered deployment of digital health technologies across the industry.

Thank you for the opportunity to comment on the Health Technology Ecosystem RFI. We would be pleased to discuss any of the above in greater detail at any time. If you have questions, please feel free to contact me at 201-370-8279 or Sarah.Lechner@hmhn.org.

Sincerely,

Sarah Lechner
SVP, Chief of External Affairs